



AGERITE PHARMACY & WELLNESS CENTER

Compounded Peptide Prescribing Guide

Clinical Reference for Licensed Healthcare Providers

• INJECTABLE FORMULATIONS • TOPICAL FORMULATIONS • SUBLINGUAL FORMULATIONS

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PROVIDER NOTICE

This guide is intended solely for licensed healthcare professionals. Information provided is for clinical reference purposes only and does not constitute medical advice. Prescribers are responsible for determining the appropriateness of therapy based on individual patient needs and applicable laws and regulations. Many formulations described herein are prescribed off-label; clinical judgment and current evidence should guide all prescribing decisions.

Why Providers Choose AGERite

503A Licensed Compounding Pharmacy

- ✓ Sterile & Non-Sterile Compounding
- ✓ Custom Formulations Available
- ✓ Direct Pharmacist Consultation
- ✓ Electronic Prescribing Accepted
- ✓ Nationwide Shipping to Licensed States
- ✓ 1-3 Business Day Turnaround
- ✓ Dedicated Provider Support Team
- ✓ 2-Day Shipping Complimentary



Dispensing Policy & Storage

ALL PEPTIDE PRODUCTS

VIAL TYPE

Multi-dose vials. Store refrigerated products immediately upon receipt per labeling.

SHIPPING METHOD

Shipped with appropriate cold or ambient packaging. Store per product labeling upon arrival.

SHIPPING FEE

2-day shipping complimentary. Overnight available for additional fee.

REFRIGERATED SHIP DAYS

Monday–Thursday only to maintain cold-chain integrity.

NON-REFRIGERATED SHIP DAYS

Monday–Friday. Orders after 2 PM CST ship next business day.

TURNAROUND & RX

1–3 business days from Rx receipt. Valid patient-specific Rx required per 503A.



Injectable Peptides

SUBCUTANEOUS / INTRAMUSCULAR

BPC-157

Body Protection Compound

SQ / IM

TB-500

Thymosin Beta-4 Synthetic Analog

SQ / IM

GHK-Cu (Injectable)

Copper Peptide — Injectable

SQ / IM

CLINICAL USE	Tissue healing, gut mucosa restoration, tendon & ligament repair, neuroprotection, angiogenesis
MECHANISM	Works primarily through nitric oxide pathways and angiogenesis; upregulates growth factors (VEGF, EGF) and promotes local tissue repair mechanisms; derived from a protein found in human gastric juice
CONCENTRATION	2 mg/mL
TYPICAL DOSE	500–1,000 mcg (0.25–0.5 mL) SQ once daily near injury site
DURATION	4–12 weeks; extend based on clinical response
KEY COUNSELING	Rotate sites; no endocrine effects; stable, well-tolerated
◆ SUGGESTED MONITORING Clinical response assessment Pain/function scores	

CLINICAL USE	Systemic tissue repair, muscle recovery, connective tissue regeneration, angiogenesis, anti-inflammatory
MECHANISM	Promotes actin upregulation; systemic anti-inflammatory via thymosin beta-4 pathways
CONCENTRATION	2.5 mg/mL
LOADING DOSE	2–2.5 mg SQ/IM 2×/week × 4–6 weeks
MAINTENANCE	2 mg 1×/week or biweekly once stable
KEY COUNSELING	Acts systemically; no need to inject near injury
◆ SUGGESTED MONITORING Clinical response Functional improvement	

CLINICAL USE	Systemic skin rejuvenation, collagen synthesis, tissue remodeling, wound healing, neuroprotection
MECHANISM	Stimulates collagen/elastin synthesis; upregulates antioxidant enzymes; promotes angiogenesis
CONCENTRATION	6 mg/mL
TYPICAL DOSE	3–6 mg (0.5–1 mL) SQ 3–5×/week; daily for accelerated protocols
DURATION	4–12 weeks; reassess at 4 weeks
KEY COUNSELING	Rotate abdomen/thigh sites; excellent tolerability; no endocrine effects
◆ SUGGESTED MONITORING Skin quality assessment Patient-reported outcomes	

CJC-1295 + Ipamorelin SQ GHRH Analog + Selective GH Secretagogue	
CLINICAL USE	GH optimization, body composition, fat loss, lean muscle, sleep quality, anti-aging
MECHANISM	CJC-1295 binds albumin for extended half-life (~6–8 days); stimulates pituitary GH release. Ipamorelin activates ghrelin receptor (GHS-R) to produce short, acute GH pulses without raising cortisol, prolactin, or acetylcholine
CONCENTRATION	CJC-1295 100 mcg + Ipamorelin 200 mcg per 0.1 mL (1 mg / 2 mg per mL)
TYPICAL DOSE	0.1–0.2 mL SQ nightly, 30–60 min before bed, fasted
DURATION	12–16 weeks on; 2–4 week break
KEY COUNSELING	Avoid carbs/fats 1–2 hrs pre/post injection to preserve GH pulse
◆ SUGGESTED MONITORING IGF-1 baseline IGF-1 at 8–12 wks	

Ipamorelin SQ Selective GH Secretagogue — GHRP	
CLINICAL USE	GH stimulation without cortisol, prolactin, or appetite elevation; body composition, sleep, anti-aging
MECHANISM	Selective ghrelin receptor (GHS-R) agonist; stimulates pituitary GH release without activating the ACTH/cortisol axis or raising prolactin — supporting recovery and sleep without systemic hormonal side effects
CONCENTRATION	1,000 mcg/mL · 10 mL vial
TYPICAL DOSE	200–300 mcg SQ nightly, 30 min before sleep
DURATION	12–16 weeks; combine with CJC-1295 for synergy
KEY COUNSELING	Most selective GHRP; no significant appetite stimulation at standard doses
◆ SUGGESTED MONITORING IGF-1 baseline & 8 wks Body composition	

Sermorelin SQ GHRH(1-29) Analog	
CLINICAL USE	Adult GH deficiency, body composition, sleep quality, bone density support, longevity
MECHANISM	Synthetic analogue of the first 29 amino acids of GHRH; binds pituitary GHRH receptors to stimulate endogenous, pulsatile GH release while preserving somatostatin feedback — closely mirrors natural GH rhythms
CONCENTRATION	1,000 mcg/mL · 10 mL vial
TYPICAL DOSE	200–500 mcg SQ nightly at bedtime, fasted
DURATION	3–6 months; may continue at maintenance dose long-term
KEY COUNSELING	Most physiologic GHRH analog; preferred for GH axis sensitivity
◆ SUGGESTED MONITORING IGF-1 baseline & 8–12 wks Sleep quality assessment	

Fasting glucose if indicated

Body composition

Tesamorelin

SQ

Stabilized GHRH Analog

CLINICAL USE

Visceral adiposity reduction, metabolic syndrome, cognitive support, off-label GH deficiency

MECHANISM

Stabilized GHRH analog; stimulates GH release; selectively reduces visceral fat via IGF-1 pathway

CONCENTRATION

3 mg/mL · 5 mL & 10 mL vials

TYPICAL DOSE

1–2 mg (0.33–0.67 mL) SQ once daily into abdomen; rotate sites

DURATION

26-week cycles; reassess. FDA-approved (Egrifta) for HIV-associated lipodystrophy.

KEY COUNSELING

Most potent GHRH analog. Caution with glucose intolerance or uncontrolled DM.

SUGGESTED MONITORING

IGF-1 baseline & q3 months

Fasting glucose/HbA1c

Waist circumference

DSIP (Injectable)

SQ

Delta Sleep-Inducing Peptide

CLINICAL USE

Sleep disorders, insomnia, HPA axis/cortisol modulation, pain relief, circadian rhythm support

MECHANISM

Modulates delta-wave sleep via hypothalamic pathways; reduces ACTH and cortisol; antioxidant activity

CONCENTRATION

1,000 mcg/mL · 10 mL vial

TYPICAL DOSE

100–300 mcg SQ, 30–60 min before bedtime

DURATION

2–4 weeks continuous; 1–2 week rest; repeat as needed

KEY COUNSELING

No known dependence or tolerance; well-tolerated at standard doses

SUGGESTED MONITORING

Sleep quality (subjective)

Cortisol if indicated

PT-141 (Bremelanotide)

SQ

Melanocortin Agonist — Sexual Function

CLINICAL USE

HSDD in women (FDA-approved); erectile dysfunction and low libido in men (off-label)

MECHANISM

Central melanocortin receptor agonist (MC3R/MC4R); activates CNS sexual arousal pathways

CONCENTRATION

2 mg/mL · 10 mL vial

TYPICAL DOSE

0.5–1 mL (1–2 mg) SQ abdomen or thigh, 45 min before activity

DURATION

As needed; max 1×/24 hrs; max 8 doses/month; do not exceed 2 mg/dose

KEY COUNSELING

AEs: nausea, flushing, transient BP elevation. Avoid in CV disease.

SUGGESTED MONITORING

Blood pressure

Patient-reported outcomes

GI tolerability

AOD-9604

SQ

GH Fragment 177–191 — Lipolytic Peptide

CLINICAL USE

Targeted fat loss, visceral adiposity reduction, fat metabolism support; osteoarthritis support (off-label); no insulin resistance or IGF-1 elevation

MECHANISM

Synthetic fragment of hGH amino acids 177–191; stimulates β -3 adrenergic receptors in adipose tissue to promote lipolysis and inhibit lipogenesis without full GH hormonal effects

CONCENTRATION

1 mg/mL

TYPICAL DOSE

250–500 mcg (0.25–0.5 mL) SQ once daily, fasted; morning preferred to align with cortisol-mediated fat mobilization

DURATION

8–12 weeks; 4–6 week rest between cycles. May combine with GH secretagogues for complementary metabolic signaling.

KEY COUNSELING

Administer 30–60 min before first meal. Does not suppress endogenous GH, raise IGF-1, or affect fasting glucose. Well-tolerated; mild injection-site reactions possible.

♦ SUGGESTED MONITORING

Body weight & composition

Waist circumference

Fasting glucose (baseline)

IGF-1 (not expected to elevate)

NAD+ — Nicotinamide Adenine Dinucleotide

IM / SQ

Coenzyme · Cellular Energy, DNA Repair & Longevity

CLINICAL USE

ATP synthesis, mitochondrial function, DNA repair, cognitive enhancement, addiction recovery, longevity

MECHANISM

Essential coenzyme in redox reactions; activates sirtuins, PARP enzymes; supports NAD+/NADH ratio

CONCENTRATION

100 mg/mL

IM LOADING

100 mg (1 mL) IM daily × 4–10 days, then 2–3×/week maintenance

SQ PROTOCOL

50–100 mg SQ daily; slower absorption, generally better tolerated

KEY COUNSELING

IM: inject slowly over 1–2 min. Transient flushing, nausea, chest tightness may occur. Do not mix.

♦ SUGGESTED MONITORING

LFTs (prolonged use)

Energy & cognition (subjective)

Metabolic panel

Wolverine Mix — BPC-157 + TB-500

SQ / IM

Comprehensive Tissue Repair Combination

Combines localized tissue repair (BPC-157) with systemic anti-inflammatory and regenerative action (TB-500). Ideal for musculoskeletal injuries, post-surgical recovery, and performance athletes.

CONCENTRATION

BPC-157 2 mg/mL · TB-500 2.5 mg/mL

TYPICAL DOSE

0.5–1 mL SQ/IM once daily or 5×/week

DURATION

4–8 weeks; reassess and extend if warranted

KEY COUNSELING

Rotate injection sites; store per labeling; discard per BUD on label

Glow Peptide Mix

SQ / IM

GHK-Cu + BPC-157 + TB-500

CLINICAL USE

Systemic rejuvenation: collagen/skin quality, tissue repair, and anti-inflammatory action

CONCENTRATION

GHK-Cu 6 mg/mL · BPC-157 2 mg/mL · TB-500 2.5 mg/mL

TYPICAL DOSE

0.5–1 mL SQ/IM once daily or 5×/week

DURATION

4–8 weeks; reassess and extend as indicated

KEY COUNSELING

Rotate injection sites; discard per BUD on label

♦ SUGGESTED MONITORING

Skin quality

Functional recovery

SELECTED REFERENCES — INJECTABLES

Sikiric P, et al. Brain-gut Axis and Pentadecapeptide BPC 157. *Curr Neuroparmacol*. 2016;14(8):857–865.

Goldstein AL, et al. Thymosin beta4: a multi-functional regenerative peptide. *Expert Opin Biol Ther*. 2012;12(1):37–51.

Teichman SL, et al. Prolonged stimulation of GH and IGF-1 secretion by CJC-1295 in healthy adults. *J Clin Endocrinol Metab*. 2006;91(3):799–805.

Roubenoff R, et al. Tesamorelin and visceral fat reduction in adults. *J Clin Endocrinol Metab*. 2010;95(9):4291–4304.



Topical Peptide Formulations

TRANSDERMAL ADMINISTRATION

GHK-Cu Cream		GHK-Cu + Niacinamide Cream		GHK-Cu + Bimatoprost Foam	
Copper Peptide 2%		Copper Peptide + Vitamin B3		Hair Restoration Foam	
CLINICAL USE	Skin rejuvenation, collagen & elastin synthesis, wound healing, anti-inflammatory, follicle stimulation	CLINICAL USE	Advanced skin repair, hyperpigmentation reduction, barrier restoration, pore refinement, rosacea, anti-aging	CLINICAL USE	Androgenic alopecia, hair thinning, follicle stimulation, eyebrow and lash density
MECHANISM	Stimulates collagen production; activates antioxidant enzymes; promotes wound healing via copper-dependent pathways	MECHANISM	Niacinamide enhances copper peptide penetration; supports ceramide synthesis; inhibits melanosome transfer	MECHANISM	Bimatoprost (prostaglandin analog) extends anagen phase; GHK-Cu promotes follicle repair and angiogenesis
STRENGTH / SIZE	2% (2 mg/mL) · 30 mL	STRENGTH / SIZE	GHK-Cu 2% + Niacinamide 2% · 30 mL	STRENGTH / SIZE	GHK-Cu 0.5% + Bimatoprost 0.03% · 30 mL Foam
ADMINISTRATION	Apply thin layer 1–2×/day; massage to absorbion	ADMINISTRATION	1–2×/day face/neck; under SPF (AM) or alone (PM)	ADMINISTRATION	Apply to scalp nightly; part hair; apply directly to skin; do not rinse
KEY COUNSELING	Avoid eyes/mucous membranes; may cause mild transient skin discoloration	KEY COUNSELING	Synergistic for dermal remodeling; ideal post-procedure protocol	KEY COUNSELING	May darken skin with prolonged contact; avoid eye contact

SELECTED REFERENCES — TOPICALS

Pickart L, Margolina A. Regenerative and Protective Actions of the GHK-Cu Peptide. *Int J Mol Sci*. 2018;19(7):1987.

Bimatoprost for alopecia areata: open-label pilot study. *J Am Acad Dermatol*. 2018;78(3):570–572.



Sublingual Troches

ORAL / SUBLINGUAL · NON-INJECTABLE

DSIP + Oxytocin Troches	
Delta Sleep-Inducing Peptide + Oxytocin · Sublingual	

CLINICAL USE	Sleep optimization, HPA axis modulation, cortisol normalization, stress/anxiety reduction, emotional regulation, mood support	DSIP DOSING	0.5–2 mg (500–2,000 mcg) per troche (prescriber-directed)	♦ SUGGESTED MONITORING Sleep quality (subjective) Cortisol if indicated Patient-reported outcomes
MECHANISM	DSIP modulates delta-wave sleep via hypothalamic pathways; oxytocin activates CNS bonding/trust circuits and supports parasympathetic tone	OXYTOCIN DOSING	100–300 IU per troche (prescriber-directed)	
FORMULATION	Sublingual troche — rapid mucosal absorption; preferred for needle-averse patients	ADMINISTRATION	1 troche under tongue 30–60 min before bed; allow full dissolution; avoid food/drink 15 min prior	
		DURATION	Nightly; 2–4 week cycles with reassessment; may continue nightly at provider discretion	
KEY COUNSELING				Store in cool, dry location; no chewing or swallowing; may use as adjunct to injectable protocols

Ordering & Delivery

● How to Order

- › Complete the AGERite Peptide Prescription Order Form available upon request
- › Submit Rx or order form via fax to (281) 605-1912 or email compounding@AGERitePharmacy.com
- › Electronic prescriptions accepted — see Rx routing below
- › All Rx must include: patient name, DOB, medication, sig, quantity, refills, prescriber NPI & license
- › Designate responsible party for payment
- › Designate delivery address on order form
- › Provider/practice responsible: credit card on file required

Electronic Rx Routing:
NPI: 1194430892 • NCPDP: 5939927

● Shipping & Delivery

- › Turnaround: 1–3 business days from Rx receipt
- › 2-day shipping complimentary; overnight for additional fee
- › Refrigerated items: Ship Mon–Thu only
- › Non-refrigerated: Ship Mon–Fri
- › Cut-off for in-stock inventory: By 2 PM CST → next business day; after 2 PM → following business day
- › After 2 PM Thursday (refrigerated): Ships Monday
- › Note: Most peptides are made-to-order and may require additional lead time. Please plan accordingly.

● Business Hours

- › Monday: 10:00 AM – 4:00 PM
- › Tuesday: 10:00 AM – 4:00 PM
- › Wednesday: 10:00 AM – 4:00 PM
- › Thursday: 10:00 AM – 4:00 PM
- › Friday: 10:00 AM – 4:00 PM
- › Saturday & Sunday: Closed

● Compliance Notes

- › All products compounded under 503A regulations — patient-specific Rx only
- › Store all products per labeling upon receipt
- › Discard per beyond-use date (BUD) on product label
- › Contact pharmacy for custom formulations or concentrations not listed
- › Full dosing protocols: [AGERitePharmacy.com](https://www.ageritepharmacy.com)

Injectable — Repair & Recovery			Injectable — GH Axis & Sleep			NAD+ — Longevity & Cellular Energy			Topicals & Troches		
BPC-157	5 mL	\$125	CJC-1295 + Ipamorelin	5 mL	\$150	10 mL \$90 30 mL \$250 50 mL \$400 60 mL \$500 90 mL \$735 120 mL \$975 100 mg/mL · IM or SQ	GHK-Cu Cream		30 mL	\$100	
BPC-157	10 mL	\$175	CJC-1295 + Ipamorelin	10 mL	\$200		GHK-Cu + Niacinamide		30 mL	\$150	
TB-500	5 mL	\$125	Ipamorelin	10 mL	\$150		GHK-Cu + Bimatoprost Foam		30 mL	\$150	
TB-500	10 mL	\$135	Sermorelin	10 mL	\$200		DSIP + Oxytocin Troches 30 troches \$55				
GHK-Cu (Injectable)	5 mL	\$110	Tesamorelin	5 mL	\$130		DSIP 0.5–2 mg • Oxytocin 100–300 IU • prescriber-directed				
GHK-Cu (Injectable)	10 mL	\$160	Tesamorelin	10 mL	\$250						
Wolverine Mix	5 mL	\$135	DSIP	10 mL	\$150						
Wolverine Mix	10 mL	\$185	PT-141 (Bremelanotide)	10 mL	\$150						
Glow Peptide Mix	5 mL	\$150	AOD-9604	5 mL	\$175						
Glow Peptide Mix	10 mL	\$175	AOD-9604	10 mL	\$250						

Partner With **AGErite**

- › Peptide selection & protocol guidance
- › Dosing strategies for complex patients
- › Customized formulation development
- › Stability and compatibility consultations
- › Patient-specific therapy planning
- › Staff training & provider education resources

Prescribing **Information**

ELECTRONIC RX ROUTING

NPI: 1194430892

NCPDP: 5939927

Fax: (281) 605-1912

Email: compounding@AGEritePharmacy.com

Clinical **Support**

Questions about dosing, formulation customization, or patient-specific prescribing?

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